

Monitoring testicular cancer (surveillance)

If the risk of testicular cancer coming back is low, your doctor may suggest monitoring (surveillance) instead of having treatment, after surgery.

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What is monitoring?

If you have [stage 1 testicular cancer](#) (early stage) with a low risk of it coming back after [orchidectomy](#), your doctor may recommend monitoring, sometimes called surveillance, instead of further cancer treatment. This is when you have regular checks for signs of the cancer coming back (recurrence).

Monitoring aims to find any signs of cancer early, when it is easier to treat and cure. This means you avoid having [adjuvant treatment](#), such as chemotherapy, which you may not need. You only have treatment if your [tumour marker levels](#) increase, or if scans show the cancer has come back.

Only a small number of testicular cancers come back. If it does come back, finding it early means there is still a high chance of curing it. But you may need a longer course of chemotherapy.

Your doctor and nurse will give you all the information you need to help you to decide if surveillance is right for you.

We have more information about [making decisions about your cancer treatment](#).

What does monitoring involve?

Your specialist doctor will tell you what kind of monitoring you will have. It usually involves:

- regular [tumour marker](#) checks and blood tests
- regular [chest x-rays](#)
- occasional [CT](#) or [MRI](#) scans
- a physical examination of your body (including checking the other testicle).

Your doctor will ask you how you have been feeling, and about any new or ongoing symptoms. Tell them if you are having [emotional](#) or [sexual difficulties](#).

How long does monitoring last?

You usually need regular clinic appointments for several years. As time passes, the risk of the cancer coming back reduces. That means your appointments and tests will become less often.

It is important to go to your surveillance appointments. If your address changes, make sure the hospital knows your new address. It is also important to tell your doctor if you get any new symptoms or feel unwell between appointments. You can arrange an earlier clinic appointment if you need to. Some appointments can happen by telephone in the first instance. The sooner a possible recurrence is diagnosed, the easier it is to treat.

About our information

References



Below is a sample of the sources used in our testicular cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

The British Association of Urological Surgeons (BAUS) Guidelines on Testicular Cancer. March 2015. Available from: https://www.baus.org.uk/professionals/sections/testicular_cancer.aspx (accessed April 2022).

European Association of Urology (EAU) Guidelines on Testicular Cancer 2022. Available from: <https://d56bochluxqnz.cloudfront.net/documents/full-guideline/EAU-Guidelines-on-Testicular-Cancer-2022.pdf> (accessed April 2022).

European Society for Medical Oncology (ESMO). Testicular seminoma and non-seminoma:

ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Last updated 2022.

European Society for Medical Oncology (ESMO). Testicular seminoma and non-seminoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Last updated 2022. Available from: [https://www.annalsofoncology.org/article/S0923-7534\(22\)00007-2/fulltext](https://www.annalsofoncology.org/article/S0923-7534(22)00007-2/fulltext) (accessed April 2022).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 May 2022 | Next review: 01 May 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help